

Willpower or Wegovy? Predictors of attitudes toward GLP-1 obesity medications

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Methods

Data were drawn from the Pew Research Center’s *American Trends Panel* (Wave 142; $n \approx 6,100$) (Tyson & Kikuchi, 2024). Survey-weighted hierarchical logistic regressions were used to predict (1) perceiving GLP-1s as a good option for people with obesity (*support*) and (2) believing these medications will reduce national obesity in the long-term (*optimism*). Step 1 included demographics. Step 2 added participants’ familiarity with GLP-1s. Step 3 added weight loss belief clusters (*k*-means on two composite scores: average ratings of perceived importance of personally controllable factors [diet/exercise] and social/biological factors [genetics, stress, grocery store access] for weight loss; *k* determined via elbow method; acceptable Jaccard stability coefficients (.74-.97); (“Mixed Attribution” cluster as reference) with willpower included as a separate predictor.

Discussion

Information exposure was the strongest predictor of support and optimism towards GLP-1s, but demographic and belief-based differences may also influence attitudes. Lower support among lower-income respondents suggests a target for outreach; future work should test whether support relates to access. Given men’s and lower-SES individuals’ lower prescription rates (Kim et al., 2025), higher discontinuation (Do et al., 2024), and higher obesity-related morbidity (Young et al., 2016), these groups may be targets for educational messaging and policy intervention. Further research should clarify how beliefs and demographics influence attitudes towards GLP-1s, to inform public education, policy and communication strategies.

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Introduction

Obesity is a chronic, relapsing disease associated with numerous health complications (McGowan et al., 2025). Glucagon-like Peptide-1 receptor agonists (GLP-1s) are a safe and effective treatment for obesity (McGowan et al., 2025). However, uptake of these medications remains low (Kim et al., 2025), often attributed to prohibitive cost and stigma (Post & Persky, 2024). Public attitudes regarding GLP-1 medications may influence uptake and adherence. Previous studies have uncovered gender and income differences in knowledge of and attitudes toward GLP-1 medications (Abutaima et al., 2024), but little is known about whether beliefs regarding factors contributing to weight loss (e.g. diet, exercise, genetics, willpower, etc.) also shape these attitudes. This exploratory study examined whether demographics, familiarity with GLP-1s, and beliefs about weight loss predict support for GLP-1 use in people with obesity and optimism toward the medications potential impact on obesity nationwide.

Results

Having heard “a lot” about GLP-1s predicted both support (OR = 2.26, $p < .001$) **and optimism** (OR = 1.33, $p < .001$). **Women** (OR = 1.23, $p = .021$) and **higher-income participants** (OR = 1.63, $p < .001$) **were more supportive** but not more optimistic. **Emphasis on willpower** as sufficient for weight loss & maintenance was **linked to lower support** (OR = 0.81, $p = .039$). The **“Dual Attribution” weight loss belief cluster** (respondents who rated both personally controllable factors and social/biological factors as highly important for weight loss) was **uniquely predictive of optimism** towards the long-term impact of GLP-1s (OR = 1.47, $p = .009$). Beyond demographics alone, adding information exposure (step 2) modestly improved model fit for both outcomes ($\Delta R^2 = .07, .01$; lower AIC). Adding weight loss belief clusters and willpower (step 3) produced a small improvement only for optimism ($\Delta R^2 = .01$; lower AIC).

K-means Algorithm Identified Four Distinct Weight-Loss Belief Clusters

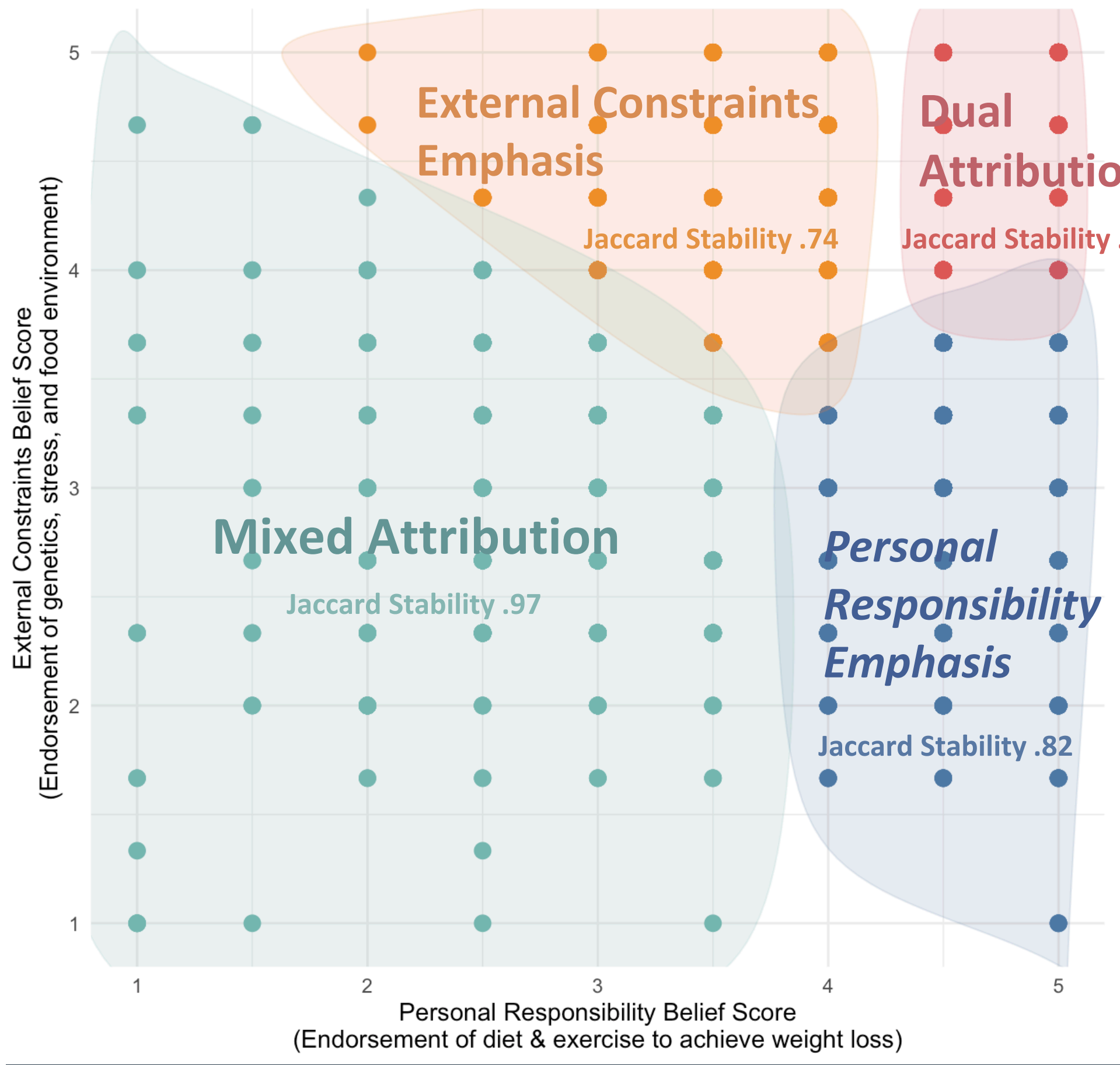
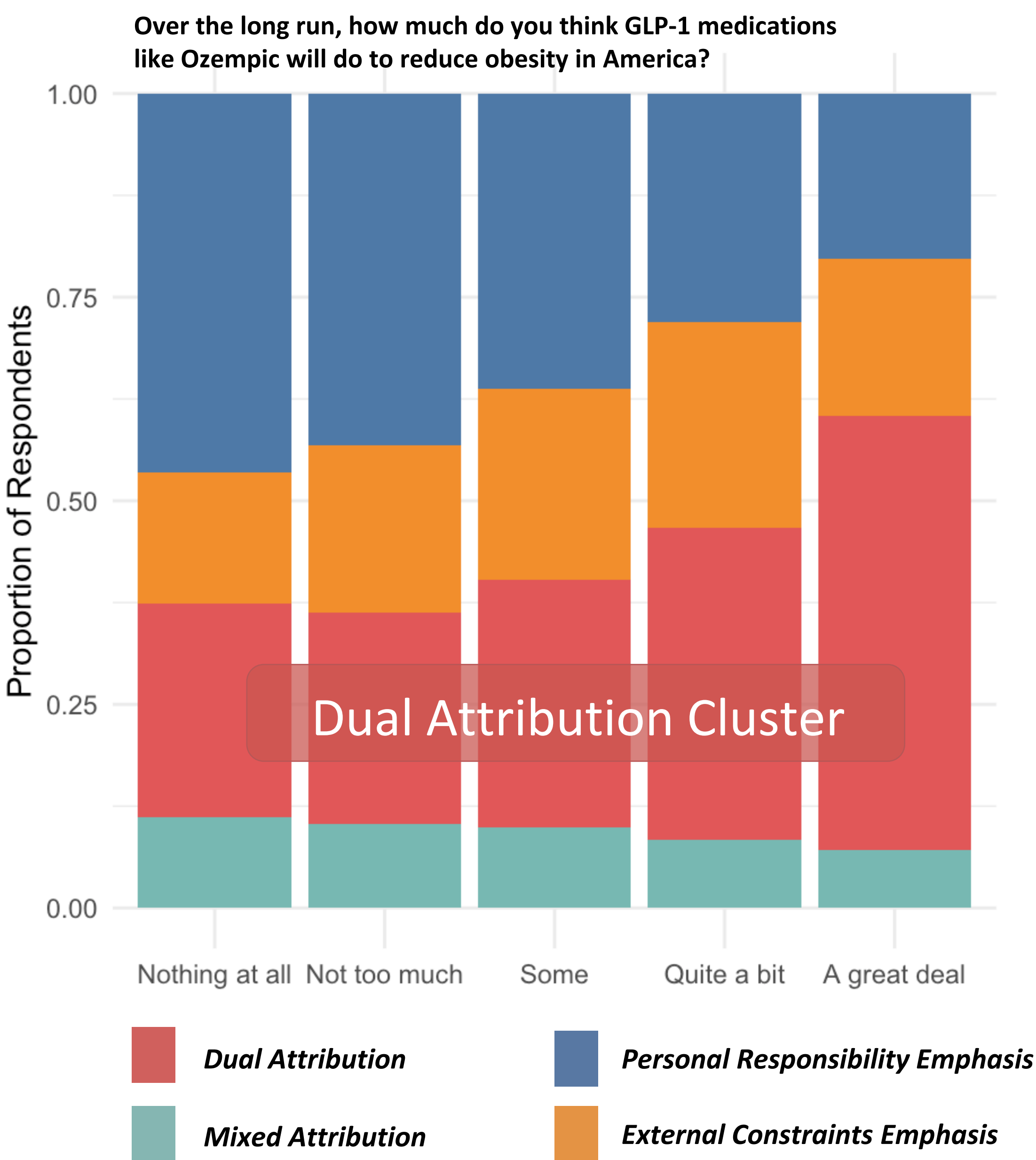


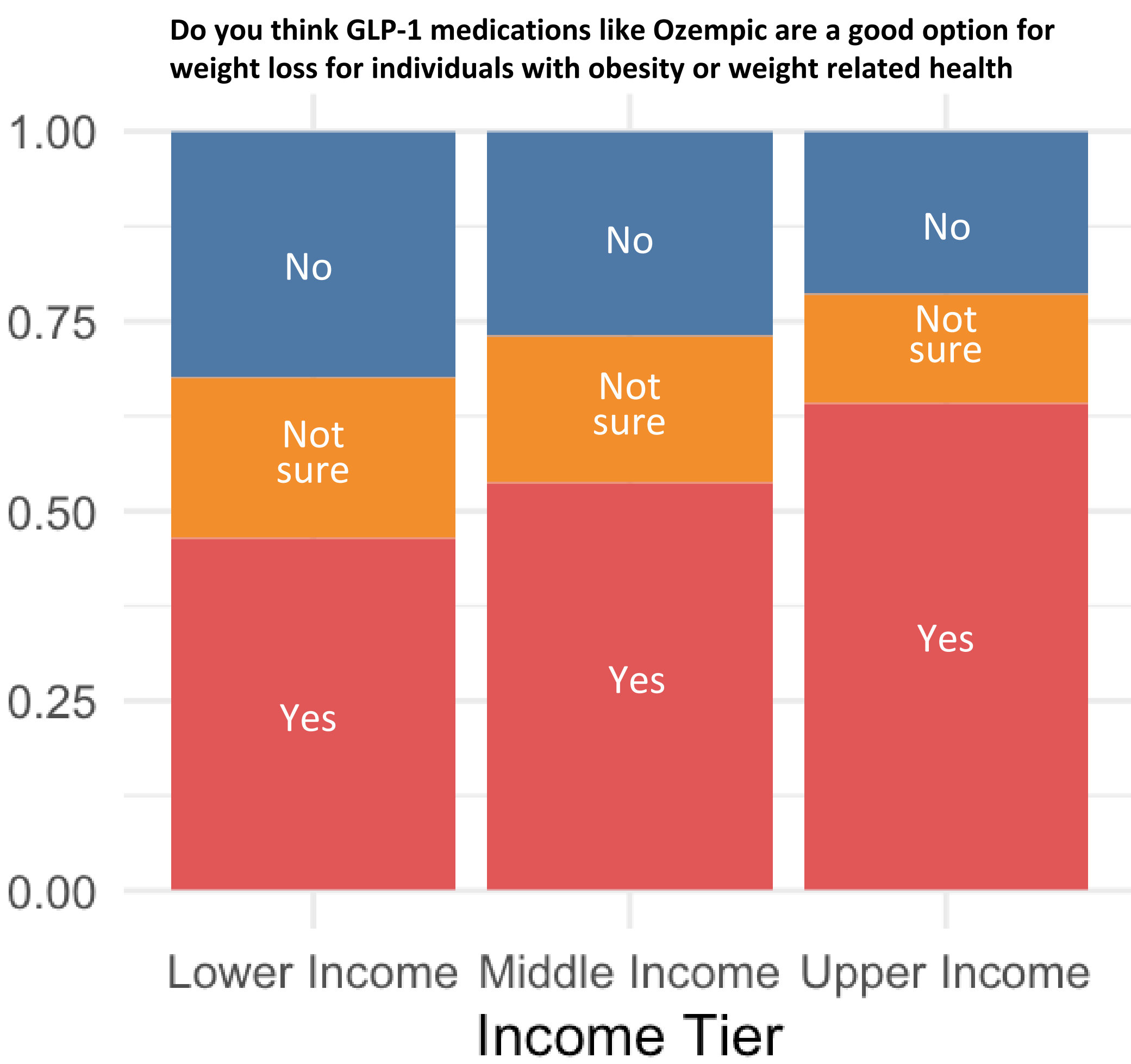
Figure. K-means clustering identified four distinct weight-loss belief patterns based on endorsement of personal responsibility (diet/exercise) to achieve weight loss and external constraints (genetics, stress, food environment) as barriers to weight loss. These belief clusters were used to examine how attribution styles relate to optimism toward GLP-1 medications.

- **Personal Responsibility Emphasis:** More men (57%), higher income, and more politically conservative.
- **External Constraints Emphasis:** More women (65%), more liberal, and most likely to endorse that willpower alone is insufficient for weight loss (80%).
- **Dual Attribution:** More women (60%), more liberal, and most likely to report hearing “a lot” about GLP-1 drugs like Ozempic (35%).
- **Mixed Attribution:** Lower income and education; least exposure to GLP-1 drugs (21% heard “a lot,” 28% heard only “a little”).

Weight-Loss Belief Clusters and Optimism Toward GLP-1 Medications



Support for GLP-1 Medications by Income Tier



Pew Research Center bears no responsibility for the analyses or interpretations of the data presented here. The opinions expressed herein, including any implications for policy, are those of the author and not of Pew Research Center.

Supplementary Tables, Figures & Survey Codebook



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